

Hair Loss - Female (Child)

Algorithm 1102 | Child (12-17 years) | Gender at birth: Female

Definition

- Hair loss assessment decomposed from NHS.UK's published when-to-get-help guidance.
- Child patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What is the person's exact age and sex, are they pregnant/recently postpartum, and when/how quickly did hair loss begin? This protocol starts at age 12; younger children require an approved pediatric route.
2. [OPEN_TEXT] Is it patchy, all over, or a receding pattern?
3. [OPEN_TEXT] Is there scalp pain, redness, swelling, pus, blistering, chemical/thermal exposure, new medicine, fever, weight loss, restrictive eating, endocrine symptoms, pregnancy/postpartum change, hair pulling/traction, distress/self-harm concern, or a safeguarding concern?
4. [CAREGIVER_OBSERVATION] What change has a safe parent or caregiver observed in behaviour, alertness, eating or drinking, sleep, school or play, and normal activity?

Triage and redirect questions

1. Disposition level 70

Is an inflamed, painful, scaling, crusting, or infected-looking scalp the main concern rather than hair loss itself?

The localized-rash family is not generated. Arrange prompt in-person assessment through the Qatar route approved by governance (GOVERNANCE_REQUIRED).

1. Disposition level 100

Is hair/scalp change part of a current emergency: chemical or electrical exposure, major burn, facial/airway swelling, breathing difficulty, collapse, severe infection with confusion, poisoning/overdose, severe dehydration/malnutrition, or immediate self-harm/safeguarding danger?

Hair loss alone is generally not an emergency, but the reported presentation may be secondary to a time-critical exposure, allergic reaction, infection, poisoning, malnutrition, mental-health crisis, or safeguarding threat.

1. Disposition level 50

Is the caller worried about the hair loss and would like it assessed?

NHS.UK guidance: arrange clinical review through a Qatar pathway approved by governance (GOVERNANCE_REQUIRED) if you're worried about your hair loss, to identify the cause before considering commercial hair clinic options.

Advice

Qatar emergency response for the underlying danger

- Call Qatar emergency services on 999 and do not drive. Follow the relevant exposure, burn, anaphylaxis, poisoning, severe-illness, suicide-safety, or safeguarding instructions from 999. Do not apply chemicals, attempt decontamination beyond emergency instructions, or give unprescribed medicine. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: airway or breathing change; collapse, confusion, or severe illness; chemical/burn exposure; self-harm or safeguarding danger.

Governed in-person hair-loss assessment

- Arrange an in-person assessment through the Qatar route and timing approved by governance to evaluate pattern, scalp disease, medicines, nutrition, endocrine illness, pregnancy/postpartum factors, traction/pulling, and psychosocial impact. Do not start supplements, topical medicines, or hair-loss drugs without age, pregnancy, diagnosis, interaction, and laboratory review. For an adolescent, provide private age-appropriate assessment with approved caregiver and safeguarding practice.
- Call Back If: rapid or patchy progression; scalp pain, inflammation, pus, or scarring; weight loss, restrictive eating, systemic illness, or severe distress.

First aid

- Call Qatar emergency services on 999 and do not drive. Follow the relevant exposure, burn, anaphylaxis, poisoning, severe-illness, suicide-safety, or safeguarding instructions from 999. Do not apply chemicals, attempt decontamination beyond emergency instructions, or give unprescribed medicine. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Hair loss", <https://www.nhs.uk/conditions/hair-loss/> (page last reviewed 24 January 2024)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from the cited NHS.UK pages under the Open Government Licence v3.0; it is not licensed STCC content.

Question.DispositionLevel=null marks a See More Appropriate Guideline redirect and is not a disposition row. Local extensions, destination mappings, safeguarding workflows, and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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